

Standard Operating Procedure

Internal Quality Control and Result-Release Control

«Hospital Name» — Supports AAC.7.b, AAC.7.c

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By	Quality Coordinator Name: _____ Signature: _____		
Approved By	Medical Superintendent Name: _____ Signature: _____		

1. Purpose

This SOP describes how «Hospital Name» runs internal quality control (IQC) for laboratory tests, and what happens the moment IQC fails — including when patient results may or may not be released.

2. Scope

Applies to every quantitative and qualitative test parameter performed in the laboratory.

3. Responsibility

Laboratory Technologist: runs IQC per the defined schedule and checks results before releasing any patient result for that run.

Laboratory In-Charge / Supervising Specialist: authorises any override when IQC fails, with documented justification.

4. Procedure

4.1 Running IQC

1. Run IQC for every quantitative and qualitative parameter per the defined schedule, before releasing patient results for that run. This SOP's approach to designing and interpreting IQC follows CLSI guideline C24 — see Section 6, Reference 1.
2. Where more than one measuring system is used for the same test, check comparability between systems and log the result.

4.2 When IQC fails

1. Stop reporting patient results for the affected parameter immediately.
2. Do not release results for that parameter until IQC is acceptable again, or a documented clinical override — authorised by the Laboratory In-Charge or supervising Pathologist/Biochemist/Microbiologist — is recorded with justification.
3. Open a CAPA record for the failure and track the root cause to closure.

4.3 Proficiency testing / external quality assurance

1. Enrol in PT/EQA schemes for every analyte where a formal programme exists and the laboratory performs the test.

2. Where formal PT/EQA is not practically available (unstable analyte, no national programme, or similar), use a documented alternate approach — e.g. split-sample comparison with a reference laboratory.
3. For any outlier result in a PT/EQA cycle, implement and document a corrective action.

5. Records

Supported by Records 4–9 of the AAC.7 Required Records pack.

6. Related Documents

- HCO_AAC_7_v2_REWRITE_DRAFT — AAC.7 policy
- SOP — Specimen Collection, Handling, Transport, Reporting and Disposal (AAC.6)

7. Review

Reviewed annually, or sooner after a serious IQC failure or major method change.

8. References

- Clinical and Laboratory Standards Institute. (2016). Statistical quality control for quantitative measurement procedures: Principles and definitions (CLSI Guideline C24, 4th ed.).
<https://clsi.org/standards/products/method-evaluation/documents/c24/> (Official page; full standard text requires purchase or subscription.)